SYNTHETIC DEMO DOCUMENT. Northfield Health is a fictional organization. This is not clinical, legal or payer guidance.

NORTHFIELD HEALTH

Inpatient Potassium Repletion Clinical Guideline

Document ID:	CARD-GUIDE-2024
Version:	v3.1
Authority Tier:	Tier 2 - Clinical Guideline
Document Type:	Clinical guideline
Owner:	Dr. Priya Nair, Cardiology and Critical Care Service
Approver:	Clinical Guidelines Committee
Department:	Cardiology and Critical Care
Effective Date:	February 10, 2026
Last Reviewed:	February 10, 2026
Next Review:	February 10, 2027
Status:	Current - Ratified

1. Scope

This guideline sets the minimum safety conditions for intravenous potassium chloride replacement in adult inpatients and emergency department patients.

2. Pre-infusion requirements

2.1 A baseline 12-lead ECG, or continuous telemetry, must be in place with no existing QRS widening or heart block before an intravenous potassium infusion is started.

2.2 Serum potassium must be confirmed by a laboratory result before replacement is ordered or started under a standing order.

3. Rate limits

3.1 Peripheral intravenous infusion must not exceed 10 mEq/hr.

3.2 Central line infusion must not exceed 20 mEq/hr and requires an infusion pump with a rate lock and continuous telemetry.

3.3 Concentrated potassium must never be given as an intravenous push.

4. Monitoring

4.1 Repeat the serum potassium level 4 to 6 hours after the infusion ends or per prescriber instruction.

4.2 Continue telemetry for the duration of any infusion above 10 mEq/hr.

5. Renal caution

5.1 Where renal clearance is reduced (eGFR below 30 mL/min or serum creatinine above 1.8 mg/dL) the dose must be reduced and a prescriber assessment obtained before infusion. See LAB-REF-109 for laboratory thresholds.